



your hypnobirthing handbook





Message from @birth-hood

Hi, thank you for booking your Hypnobirthing with me!

I'm Leanne, mum of two, (and a cocker spaniel, Ron) Doula, Perinatal Yoga teacher and Birth Trauma Resolution practitioner, and obviously a hypnobirthing nerd!

When you become pregnant, for the first, or the fifth time, your mindset changes instantly, as well as being excited, it can be overwhelming and full of uncertainty. The years of birth fear becomes a huge reality and you have no idea but to go with the flow? How you feel is so important, it will impact how we birth and how we feel about it after.

What better way to help you gain knowledge, tools for calm and control and shed loads of resources than Hypnobirthing?

But FYI.... Hypnobirthing doesn't involve being hypnotised; instead, it teaches you how your body works on a muscular and hormonal level when in labour and how you can use various relaxation techniques to ensure you are working with your body (rather than against it), making birth more efficient and comfortable.

I adore teaching birth preparation, supporting you and your partner as you adjust to this new phase of your journey, coming away feeling totally in control and ready for birth!

I have created this little guide to help to help you work through the course with me, and continue once the course is finished, Let's start your hypnobirthing journey now!

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Leanne xx

what is hypnobirthing?

The chances are you know what hypnobirthing is at this point. Let's sum it up below just to be sure and to help explain to partners, family or friends who may think it's a load of rubbish!

So here it is!

It's getting informed about birth- It's learning all about the process of birth, the hormones involved, how to choose your birth place, how your environment can help you to feel positive and help the process along, the role of the birth partner, the stages of labour, positions to help aid baby's position, how to write a birth plan and all sorts of other stuff that if you haven't been at loads of births (and most of us haven't) you just won't know unless you've been told it or sought it out. The more you know about birth, the more likely it is that you will feel prepared to face it yourself.

It's a change of mindset- Knowing what the body is doing and HOW can really help you to see birth as a bodily function like any other and take a lot of the fear away. Between getting as informed as possible and taking in lots of positive information about birth, you can completely change your mindset from one full of fear of what can 'go wrong' into one that expects birth to 'go right' (whatever that means). Feeling positive and confident at the start of labour can go a long way to helping the process along by keeping away from fight or flight and producing lots of oxytocin.

It's learning decision making tools- Knowing your rights when it comes to birth (psst, you can pretty much do what you want) and knowing how and where to get all the information you need, helps you to feel informed and confident in making the decisions needed to birth your baby. This may be during pregnancy, by getting information from third parties such as AIMS or doing your own research. It may be during labour, in which case using decision making tools designed to get as much information about your circumstances will help you to make decisions about your care. Just knowing the questions to ask to find out if it's safe to take 30 mins to make your decision can make a huge difference. Nobody wants to make a decision in a rush when some time could have been taken to process it and ensure the choices were understood and considered and the best one for the family chosen!

It's the hypnobirthing tools you've probably heard about- Last but not least there are the famous hypnobirthing tools, what comes to mind when people say hypnobirthing (aside from swinging clocks and vagina whispering). These tools are an incredible way to prepare for birth and remain calm and comfortable throughout. However, on their own without all the stuff above, they're not enough. Add these into your routine alongside all the information and mindset stuff and you have got this!

Breathing Techniques- These ensure you're taking lots of nice big even breaths throughout your labour to keep the oxygen flowing nicely, this will help your uterus to be able to do it's job as efficiently and comfortably as possible. They are also a great way of distracting you from what's going on around you and giving you something else to focus on.

It sounds simple but breathing techniques can do so much to keep you calm and if you practice them during pregnancy you will feel the benefit on the day.

Visualisation– This is a way to almost practise birth before its time to do it for real. Thinking in your head about how you'd like your birth to go or saying it out loud, can help you to feel more confident in your plans and trick your mind into thinking it's nothing new when labour starts for real! This can help reduce the chance of entering fight or flight and keep oxytocin flowing!

Affirmations– Using the law of repetition, you can convince your brain to believe something just by showing it the same information repeatedly. Instead of talking about how you DON'T want labour to go, write affirmations and say things about how you DO want it to go. Giving your brain a bit of cheerleading can help it to believe in you! Instead of saying 'I don't want to tear' say 'my body WILL stretch to accommodate my baby'. After a while the affirmations sink into your subconscious and help you to have confidence in yourself which in turn will help you feel more relaxed and encourage the hormones needed for labour.

Relaxation– As humans living in a world full of things to do, technology etc. we rarely spend time relaxing and doing absolutely nothing! This means that our neocortex, the part of our brain responsible for rational thinking, is almost never switched off. We very much want that part of the brain to be out of the way during labour and not interfering with the process by thinking of unhelpful scenarios! By practising relaxation during pregnancy, you can make it easier to relax on cue, a skill that's very handy for labour! Try lying back and doing absolutely nothing, on the sofa, in bed or in the bath, wherever you like! If you find it difficult to switch off then doing something that doesn't require much thought counts too! Colouring in, knitting/ crochet, walking in nature etc. may be more your thing. Do what works for you!

Hypnobirthing tracks– Listening to hypnobirthing tracks will help you to relax, they are usually written by hypnotherapists and therefore help to induce a state of calm and relaxation. The words will also provide you with lots of encouragement and self belief as they talk you through how amazing you and your body are. Don't worry if you fall asleep listening to them, they will still be going into your subconscious even if you're not actively listening. Just be sure you don't listen when driving or operating heavy machinery as this is not safe. It's fine to listen to affirmation tracks anytime though for that little boost of confidence!

Anchoring– Whilst you are practising relaxation you can help by setting up cues for your brain to associate with relaxation and therefore help you to do it whenever you want, these are called anchors. Anchors can help you to incorporate all 5 senses and therefore make the conditions seem the same wherever you are. You may spray a room spray or have an essential oil that you use whilst relaxing that you then use in labour, you may have a blanket or cushion that you have with you whilst relaxing that you can have with you in your birth space or you may use your hypnobirthing tracks or relaxing music to aid your relaxation, then you can use this during labour and it will all help you to relax despite other things being different such as the location (or the fact you're in labour eek!).

TOP TIP– Use all the hypnobirthing tools at once throughout your pregnancy and in labour and see how chilled you can feel!

lets look at the science of hypnobirthing....

As humans, we're pretty negative, we look for reasons not to do things, issues that might crop up, however unlikely.

If you think about it, this has an evolutionary function. If you were to think about walking through some woods full of predators in the pitch black, you might worry. You may think of what could happen to you that would prevent you from getting home safe. Perhaps being attacked by a bear, getting lost, tripping over and hurting yourself, falling into a ditch. It's likely that once you have a think about it, you decide the only way to prevent these horrible things is to just not go. Thus, the brain has kept you safe! Bravo brain!

However, when what you plan to do is give birth, go to a job interview or some form of public speaking, your brain is an idiot. Those things are VERY unlikely to kill you. To prevent yourself from doing something like that because of a made up sense of danger, is bloody stupid. Let's delve deeper....

The brain has billions of nerve endings called neurons. These nerve endings come together and are responsible for all our mental functions. Our brain takes in everything we see, learn and are told and creates responses using this information.

Part of our brain which is most important when talking about the science of hypnobirthing is the old brain. This is the oldest part of human brains going back 200,000 – 250,000 years. Since then our brains have developed to include far more areas. However this is what we concentrate on when talking about hypnobirthing. The old brain is there for survival. It keeps us safe. This part of our brain contains other things within itself, the limbic system which in itself contains the hypothalamus, the hippocampus, the amygdala just to name a few.

The limbic system is often referred to as our centre for emotional response, it stores our memories and is our safety response control. When we were cave people all we cared about was eating, reproducing and not being eaten by a sabre tooth tiger! Our brains were not as developed so we didn't have communication skills like we do now! We relied on our 5 senses to go about our lives. These 5 senses would help our limbic system in deciding how we felt and what we needed to do in certain situations. It would help our brain decide if we needed to set of fight or flight (our sympathetic system) or if we were safe (our parasympathetic system).

So how does this relate to hypnobirthing?

Its important to remember that our limbic system is still very active today, and whats more important is that the limbic system responds to things we personally perceive as dangerous or a threat.

We don't have as many threats as our ancestors, thankfully! But we do all have our personal things we are frightened of and for all of us this may be different, interviews, spiders, water etc. All these things you are frightened of can elicit that same response our ancestors would have had when coming face to face with a huge predator. Our brain does not realise how safe we actually are in today's society! THE IDIOT!! Even though we have now evolved, a part of our brain called the Neocortex, which is the area of the brain responsible for logical thinking. However the limbic system will override this part of our brain making us hit fight or flight. Which explains why even when you try to reason with yourself over those things you are scared of, that the little devil comes and sits back on your shoulder and says "sorry mate you're wrong GET SCARED!". Your brain is messing with you whilst trying to save you! Annoying isn't it?!

So let's look at what happens if we perceive birth as dangerous?

Part of the limbic system called the hypothalamus is responsible for the function of our autonomic nervous system, (commonly referred to as the ANS) The ANS regulates our heart rate, breathing, blood pressure and arousal in response to our circumstances and how we feel about them. Our ANS is hugely influenced by our subconscious. Our subconscious is deeply connected to our inner selves, this is what makes up our thoughts, opinions, feelings and makes up our value and belief system. Our subconscious makes us who we are, but it collects everything you've ever been told and stores them there.

Our subconscious essentially just wants to keep us safe and alive. It does this by keeping those memories and feelings and categorises them as safe (setting off your parasympathetic system) or unsafe (setting off your sympathetic system, known as fight or flight).

For example let's think of kittens, if one walked into your room right now how would you feel? Your brain would decide by looking at what you already know about kittens, memories, things you've seen before and using all that information, it will decide whether you feel safe or unsafe in the situation. Most of us would feel safe and set off the parasympathetic system and be flooded with oxytocin and feel quite happy about the random kitten in our room.

Now let's imagine that kitten is not a kitten, it's a HUGE crocodile! MASSIVE, it's 7 foot long, it's bleeding from its mouth, it's got scars EVERYWHERE! Your brain will do the same thing, it'll work out what you know about crocodiles, personal experiences, what you've been told, what you've seen on the tv and whether they look safe. It will pull all that together and decide if you feel safe or not. The likelihood is that it will tell you you are unsafe and set off the sympathetic system.

This will then set off physical reactions in your body:

- You will be flooded with adrenalin
- Your heart will beat faster
- You will start sweating
- Your oxygenated blood will go to your arms and legs ready to fight or run
- Your mouth will go dry as digestion is the last thing your brain is thinking about now.

It does ALL this stuff to save you from this threat that you have decided for yourself is dangerous.

So if you believe birth is scary, unsafe etc. , this is how it impacts your birth.

Brain thinks labour is painful:

- The ANS retrieves that information from the subconscious
- The ANS sets off the sympathetic system
- So fight or flight starts
- Sweating (dehydration)
- Breathing is fast
- Heart rate increases
- Oxygenated blood goes to the defence systems and away from the uterus
- Makes the inner layers of the uterus harder to move
- Baby pushed against hard muscle
- Adrenaline is released which impacts our hormones of labour especially oxytocin and endorphins and all of this makes labour more painful and leads to interventions

So how hypnobirthing stop this? THE SCIENCE

Hypnobirthing is simply about replacing your current beliefs with new ones. Using hypnosis you can get to your subconscious and change your views, we do this with hypnosis mp3s, by watching positive birth videos can give you positive input about birth, stay away from the negative talk of others, plus using visualisation and affirmations.

The more work you put into stopping feeling scared of birth the less likely you are to enter fight or flight. The more likely you are to have a birth you deserve to have and not get in your own way!

What words to you think of when you hear the word birth?

Where do these come from?



building a positive mindset

So we know how fear of labour and birth can affect the body physically during the birthing process, now let's look at all the things we can do in pregnancy to add positive information about birth to our subconscious and change our mindset around birth!

We take in negative thoughts on birth from what we see or hear. TV shows, movies and documentaries are a huge reason that many people fear birth. Along with horror stories from friends and family and unhelpful headlines in the media, it's not surprising that most people are not looking forward to birthing their baby!

What can we do to change our mindset into a positive one instead?

Write down all the things you can think of and then make sure to start using them to make you feel informed and excited to birth your baby.



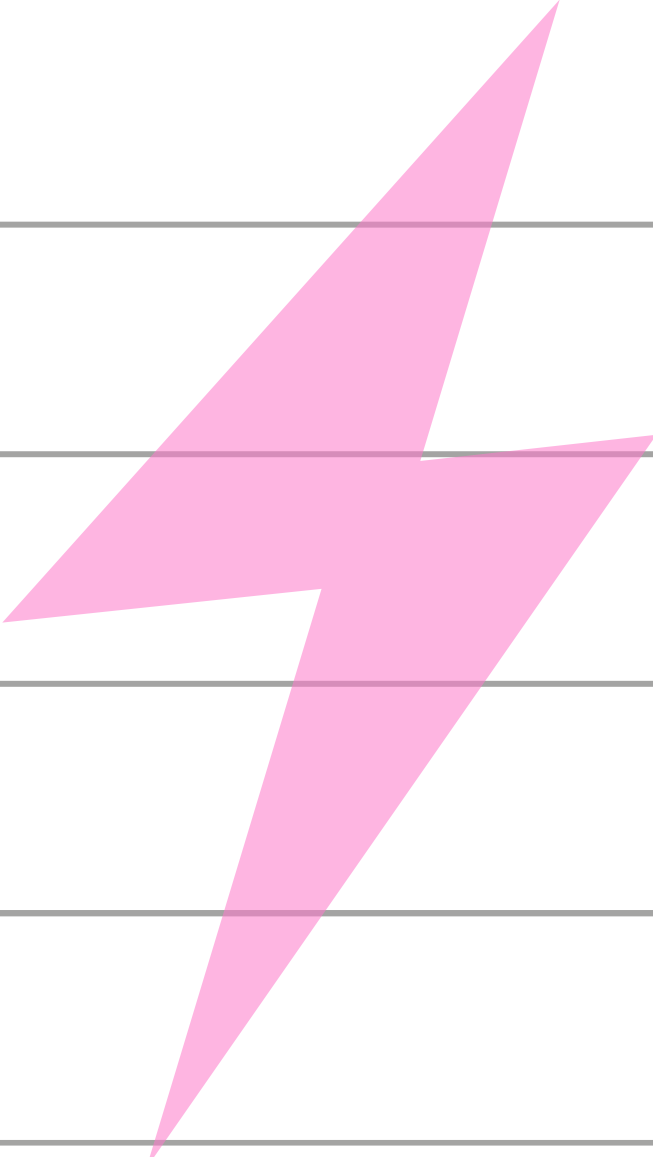
now lets work on those worries

The way we feel about birth physically impacts the way our body births. So here is a space in which you can start to let go of some of those fears.

1. List all of the things that you are worried about, the things that come up when you think of birth below. Let's get this stuff of your chest and out of your mind!
2. Once you've written them down give them a rating of 1 - 5. 1 being less worried and 5 being most worried.
3. Have your partner do the same.
4. Now look at your lists together and discuss what you can do to help yourself feel less concerned about your worries. What can you do to take the 5's - 1's.

Once you have completed this, take some time to relax and listen to your 'Fear Release' MP3. You should listen to this track from this point onwards whenever you feel any of these concerns creeping back or if you feel you want to release any new worries.

Worries or Concerns	Rating 1-5



writing your own affirmations

Affirmations are sentences created to affect the conscious and the subconscious mind so that in turn, they affect our behaviour, thinking patterns, habits, and environment. The words composing the affirmation, automatically and involuntarily, bring up related mental images into the mind, which inspire, energize and motivate. The affirmations and the resultant mental images get engraved on the subconscious mind, which in turn, changes the behaviour, habits, actions, and reactions according to the repeated words.

Example: I'm worried about how I will handle the contractions becomes "I surrender to the rhythm of birth, knowing I am capable of handling each moment gracefully"

What is your worry or concern?

Your positive affirmation

What is your worry or concern?

Your positive affirmation

What is your worry or concern?

Your positive affirmation



visualisation activity

Use these activities to get a feel for how visualisation can feel! Please don't worry if you don't nail it the first time! Practise makes perfect! You can spend time visualising your positive birth as often as you like, it's a fab way to change your mindset and send oxytocin to your baby!

1. First you can have a partner or friend listen to you for 5 mins (or record) telling your birth story as if it's already happened. Tell it in as much detail as possible, how did you feel? Who was there? Where were you? What happened when? Make it as positive as you like, this is supposed to be YOUR perfect birth. If you would like to complete this activity alone, just set a timer and get chatting. Go with the flow and see what amazing birth story you can come up with and imagine it's yours! How do you feel now? A bit silly? That's alright! Do you feel motivated to go and get that birth you were just visualising? Go and get it!

2. Write your birth story in the past tense, this can be done as well as the previous activity to see which one you prefer! Some people don't find it easy to visualise. Read through your finished story and see how you feel! Does it make you feel like your birth could be as positive as that? Are you feeling really pumped about it? It's working!

3, Create a vision board to help you based on the key points from your story (e.g., fairy lights, birth pool, your partner etc.

Notes:



breathing techniques & MP3

Practise these for 5 mins daily until you can do each one easily without thinking, this will make them easier to use on demand when you need them.

Chilled Out Breathing

- In for 4, out for 8. Do this for at least 3 minutes for it to take effect but keep it up as long as you want! Use this anytime you feel anxious, stressed or panicked. This could be anything at all, you really can't use this too much so if in doubt, try it out! It's super handy for on the journey to or whilst waiting for antenatal appointments and scans. Visits from annoying family members, traffic jams, you name it! Try it on the way to your birth place to keep nice and calm!

Wave Breathing

- In for 7, out for 7. (or just LONG AND SLOW) This is all about making sure you are breathing in nice and deeply and steadily. It can give you something to focus on during surges and ensure you're getting plenty of oxygen to keep that uterus working comfortably and efficiently like a badass!

Birth breathing

- In for 1, out for 7 (try with a humming noise as you breathe out, feel it in your belly). This is for the birthing phase of labour to help your baby move down and out! You are likely to find that you do this automatically at this phase but if not, actively doing it can help if you're perhaps holding your breath and holding back.

MP3s- This is a rough idea of when you might find them useful but please do listen to the ones that suit you best when you fancy it! The idea is to use them regularly to chill out and then make it more and more easy for you to relax on cue, very helpful in labour! Do remember that you don't have to be awake or actively listening to each one for it to have an effect. Do not listen whilst driving.

TBU Relaxation- This is a general relaxation track which is useful to birthers and birth partners alike! It's useful to listen to this one daily to get you used to using it to relax. You may find that if you pop it on as you go to bed, you start falling asleep closer and closer to the beginning!

TBU Birth Run Through- This should be used once or twice a week throughout pregnancy to help you visualise your birth and have positive thoughts around birth. Take those with you on the day! You can then, as your estimated due date approaches or if you go post dates, increase this to daily where possible.

TBU Confidence- Use this a couple of times a week throughout pregnancy for a confidence boost. It can be used more often if you feel it would help. It may be useful before antenatal appointments to give you confidence to speak up for what you want!

TBU Fear Release- Use this together with your birth partner as early on as possible in pregnancy to help ease any fears about birth. It's a great one to use if you're feeling fearful or anxious about anything at all. It may be useful if circumstances change during your pregnancy which may change your birth plan.

TBU Affirmations- Use this for a little boost of confidence anytime you feel like. Try to build it into your daily routine because we all deserve to hear how amazing we are on a daily basis! This can just play as you're going about your day, perhaps getting ready in the morning.

other important stuff

Pelvic Floor Exercises

- You do these twice a day, every day. No excuses. This is something that can make sure your life and your sex life stay happy and healthy and absolutely no wetting your pants! It's just as important to focus on the release too- we need a floppy pelvic floor for birth! Do quick ones (think 'on-off') longer ones holding for 3 seconds then releasing and then longer ones for 10 seconds or more. Repeat a few times.

Perineal massage

- Practice this daily from 36 weeks but only if you fancy it. Some people swear by it to reduce tearing and others don't enjoy it one bit and would rather not. You do you!

Massage

- Encourage your birth partner to practice various massage techniques regularly so that they are a pro by the time you're in labour, these can be used to increase your oxytocin both in pregnancy and in labour and help you feel more comfortable.

Prenatal bonding

- Take some time to sit and chat to your baby, rub your bump, sing or think about bringing your baby home. All of this will increase your oxytocin and send it to baby as well, helping them to feel safe and calm.

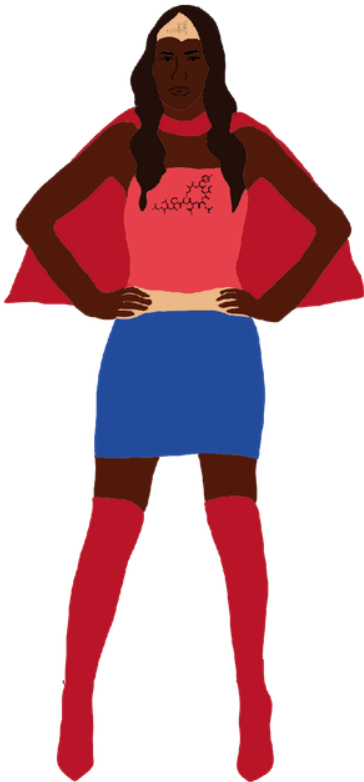
Biomechanics & Yoga

- Encourage your body to move mindfully and explore gentle yoga poses regularly throughout pregnancy. Practicing alignment, stretches, and strengthening exercises now helps your muscles, joints, and pelvis work efficiently during labour. These movements can reduce discomfort, support optimal fetal positioning, and help you feel strong, balanced, and confident.

Notes:



hormones – the birth heroes



Oxytocin– The Ultimate badass

Powers– Fills you with love, makes you feel euphoric, contracts your uterus, tells your breasts to release milk, bonds you to baby. It can do all of this in pregnancy, during labour/birth and after!

Where to find– Anywhere there's laughter, love, snuggles, massage, nipple/clitoral stimulation and all the good stuff! Try some chocolate and a funny movie!

Weaknesses– Hates being observed, being around strangers and unfamiliar places. VERY shy.

Everybody loves Oxytocin (except that pesky adrenaline), you want to encourage Oxytocin to show her face as much as you can!

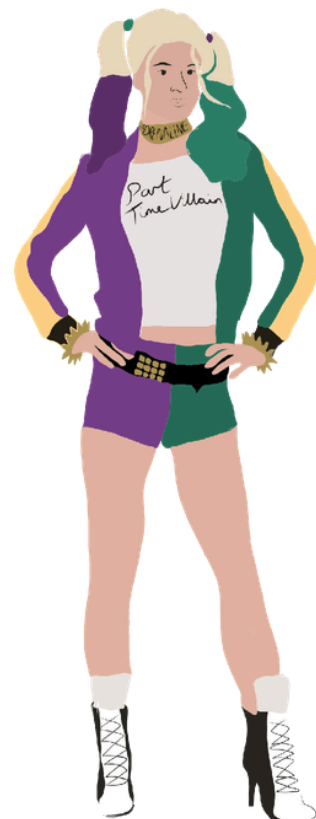
Adrenaline– The Part Time Villain

Powers– Makes you alert, blocks oxytocin, increases heart rate and gets ready for fight or flight.

Where to find– In places with fear or anxiety, and at the end of labour where she decides to do some good! Love her or hate her she has her uses and she's not always evil!

Weaknesses– Can't help but get involved when they think you're not feeling safe and calm.

Uh Oh! When it's not the right time, it's presence will slow or stop birth making the process longer and less comfortable.



hormones – the birth heroes



Melatonin- The Sidekick

Powers- Uses the power of darkness to boost her friend Oxytocin and help labour and bonding along.

Where to find- In quiet dark spaces with privacy. At night, when the hustle and bustle of the day has ended.

Weaknesses- Hates bright lights, screens and being watched.

TIP- Switch off the lights/ phones in your birth space!

Endorphins- The Life of the party

Powers- Can give you a buzz, a dose of pain relief 200x stronger than morphine. Makes you feel calm and in control.

Where to find- Brought on by physical contact and massage/TENS Machine. Encouraged by love and laughter.

TIP- Encourage endorphins in early labour and save them for later!



How might you amend your birth space knowing about these?

Relaxin- Ms Stretch

Powers- Makes your body bendy and flexible to help your pelvis open up to accommodate baby. Does the same for baby to help you both work together smoothly.

Where to find- All over your body throughout pregnancy!

Weaknesses- Can unfortunately lead to injury during pregnancy as things stretch a little further than usual, be careful not to lift anything too heavy!



Prolactin- The Nurturing Protector

Powers- Brain washes you to want to care for your baby no matter how tired you are, no matter what they need. Compliments oxytocin and aids bonding with baby. Encourages your body to make milk for baby.

Where to find- At its strongest at the point of birth and just after. Also during the night whilst breastfeeding.

Prostaglandins- The Hidden Helper

Powers- Softens your cervix and can bring on surges.

Where to find- Around an irritated cervix and hiding in semen!

HINT- This is what sweeps are aiming to stimulate and what is found in a synthetic form in some types of induction methods.



choosing where to have your baby

Labour Ward or Delivery suite

These are situated within hospitals, they usually have at least one birth pool and access to all pharmaceutical pain relief. There can be consultants, anaesthetists, surgeons and NICU staff for baby in attendance if necessary. This is where most inductions are carried out. This is also where someone would have to be for an assisted birth or caesarean and for continuous fetal monitoring (CTG monitoring). For this reason, this is likely to be the best place for someone who is expected to have health issues with themselves or baby during or soon after birth. Although anyone is free to use the labour ward for their birth, low risk people are more likely to have interventions when giving birth on the labour ward so may wish to consider other birth places.

Home Birth

Anyone is entitled to birth at home. Most trusts will have policies that advise against birthing at home if the birther doesn't fit the criteria. However, it is always up to the birther themselves to gain all the information they need to get a full picture and then choose where they would like to birth their baby. Birthing at home is as safe for low risk first time parents as hospital and safer than hospital for those who have had a baby before. Due to the abundance of oxytocin in your own environment and lack of need to transfer anywhere during labour, the birth process is less likely to be hindered at home, labour is more comfortable and there is a lower chance of haemorrhage there. Gas and air is available at home and you can have access to a birth pool if hired or bought personally (some trusts may have pools you can use). Pharmaceutical pain relief is limited at home, some people may have access to pethidine if they can get it prescribed by their GP to keep at home for the birth.

Birth Centre or Midwifery Led Unit

A birth centre or MLU may be located within a hospital or be a stand alone unit. These are a cross between the labour ward and home. The environment is more suited to your hormones as it is more homely and often has features such as mood lighting, a sofa or a small kitchenette. They will all vary so have a look on their website or social media accounts to find out what facilities your local one has. MLUs usually have more birth pools than labour wards and access to gas and air. For further pain relief, an instrumental delivery or caesarean there would have to be a transfer to the labour ward. As they are NHS property, not everyone can choose to birth at an MLU. MLUs have criteria that they would like birthers to meet before giving them access to the unit. If you find that you don't meet the criteria but you know there are no significant issues for you and baby, then speak with a Professional Midwifery Advocate or Head of Midwifery. These people are able to discuss your individual circumstances and can decide to overrule the policy and give you access to the unit. For example someone planning a VBAC or a couple of years older than the policy dictates someone must be to use the unit, will often be able to use the unit after having a meeting to discuss it.

birth place pros & cons

Use this space to help you work out where you want to birth your baby.

Write down your pros and cons for each birth place.

Once youve made a decision on which birth place you prefer, see what you can do to try and counteract the cons there. Is there anything you can change about the setting or help your- self feel less worried about the cons?

		pro	con
B I R T H	H O M E		
	C E N T R E		
	L A B O R		

tailoring your birth environment

Fairy Lights LED candles – Create mood lighting to promote melatonin and give a bit of a magical feel

A nice big towel or two– More comfortable than hospital ones.

A comfy blanket and pillow from home– more comfortable and make you feel more at home.

Facecloth– Used with warm or cold water to provide comfort and relief during labour.

Face Spray mist– Can be cooling and comforting during labour if birther is feeling hot.

Photos of loved ones/ pets/ special times – Helps promote oxytocin.

Playlist of music– Happy cheesy music to get you dancing, move baby down and promote oxytocin. Or background music to promote relaxation.

MP3s– Help to stay calm and promote relaxation.

Portable fan– Can help to keep cool.

Massage oil– For massage techniques, promote oxytocin and endorphins.

Essential oil– Aid relaxation if it is one that is used at home or during relaxation.

Portable blackout blind– Helps block out light and aid melatonin.

Eye mask– Keeps the birther in the dark for melatonin and help to stay in their zone on the journey.

Sweets/sugary snack– Raise blood sugar and keep energy levels up, can help if labour slows.

Fluffy socks– Keep feet warm when all your blood is directed to uterus.

Affirmations– Boost of confidence, a reminder of how strong the birther is etc.

Head massager– Can help to keep them distracted, produce oxytocin and endorphins.

Baby clothes/comforter– Helps to boost oxytocin, reminder that baby will arrive soon, reminder of how small baby is.

Vibrator – helps get your oxytocin flying if things slow down.

birth partner's role

Birth partner's role

As a birth partner you have the honour of being present as a baby is born, one of the most incredible moments you'll ever have! It's up to you to get prepared as best you can to help support a person you love as they birth their baby. You are such a bloody important part in all this! The practical and emotional support you can give and the continuity of care that you provide can do so much to help birth along the way. The way you feel about birth and your mindset will have an impact on the big day. You gotta do whatever you can to feel confident in your ability to support birth and take an active role. This will mean that the birth space is full of positive energy and lovely and calm. You are going to work as an amazing team and look back on this for the rest of your lives! You'll have your own positive birth story to tell! Get ready to get that oxytocin flowing!

During the pregnancy

- Get informed about birth by really getting involved with your hypnobirthing learning.
- Read birth related books (check out our recommended reading list).
- Be involved in writing the birth plan so that you understand it well.
- Listen to the relaxation MP3s regularly.
- Practise the breathing techniques so you know them for you and to help use them in labour.
- Watch positive birth videos to show how calm and lovely birth can be.
- Complete the fear release exercise together.
- Do research about the things you may be anxious about for reassurance.
- Practise your massage techniques.
- Find out the practical details like directions to the birth place, parking, phone number you'll need in labour.

Use tools from the course

- Aid visualisation with prompts.
- Remind of breathing techniques/ do together.
- Use affirmations.
- Find mp3s to aid relaxation.
- Manage environment to support birthing hormones.

Oxytocin generator

- Make laugh
- Hugs
- Kisses
- Massage
- Nipple and clitoral stimulation
- Put something funny on
- Talk about funny stories/ embarrassing moments from the past.

Practical stuff

- Dry run of the hospital journey or birth space set up if home birth- know where to park and how to pay for it, what entrance to use etc.
- Help to pack bags to ensure you know where everything is when its needed
- Pack the car to travel.
- Set up birth space at home or other setting with affirmations, comfort items, lights etc.
- Do birth preference together and know what everything means and why it has been chosen.
- Create a playlist or two for the day, uplifting for early labour and calming for later on.

Early Labour

- Make yummy food to give energy.
- Spend time being romantic/ cuddling as appropriate.
- Watch funny stuff together.
- Go for a walk or out for lunch.
- Do a silly dance routine (or a serious one if you're pros!).
- Keep an eye on how far apart surges are, if timing, keep the times to yourself.
- Listen to when they wish to go to hospital/ be seen by a midwife and help sort that out.

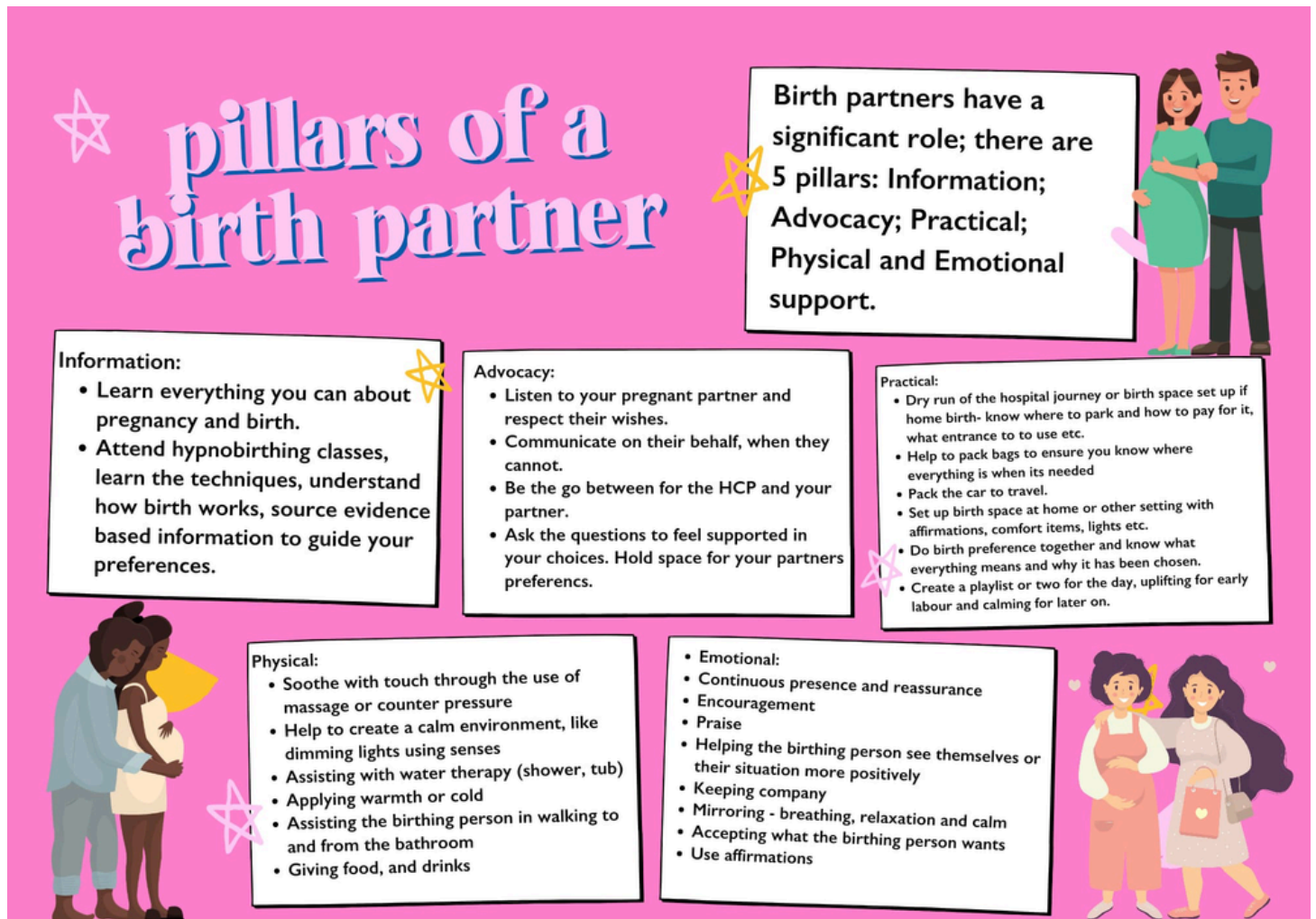
Active Labour

- When midwives arrive or you arrive at the birth setting then talk them through what's been happening so far.
- Explain birth plan A and answer any questions.
- Set up the space with everything to support the birthing hormones, blackout blind, lights, sign on the door etc.
- Manage birth environment- who's in the room? Must they be there? Are they being kind and quiet?
- Advocate with the care providers when needed, remind them of their use of language and the birth plans.
- Help with giving food and drink.
- Be in charge of the music/ mp3s.
- Suggest going to the toilet and regular position changes.
- Use cool or warm facecloths to keep comfortable.
- Look out for signs of transition and give loads of encouragement.
- Provide cheerleading when necessary.
- Help decision making using BRAIN to get all the information needed.
- Make sure the birth plans are carried out where possible or present plans B or C where needed.
- Look out for tension in the jaw, shoulders etc and help to relax.

When baby arrives

- Help to facilitate skin to skin by helping remove clothes or even helping to hold baby there
- Ensure birth preferences are adhered to with choices after birth eg. placenta, vit K etc
- Help to fetch items from bag such as clean clothes, maternity pads etc.
- Enjoy skin to skin with baby whilst birther gets themselves dressed/ uses the toilet/ has a shower etc.
- Help facilitate breastfeeding if this is what is wanted, be encouraging, do what you can to take care of the less important things around them.
- Be involved in nappy changing, getting dressed and holding baby while they sleep, this helps your bond with baby and also is a great help during recovery from birth!

Notes:



birth tools to support labour



Peanut balls are a versatile tool to help open the pelvis, encourage your baby to rotate, and support labour progress, this tool is especially helpful if you have an epidural or need to rest in bed.

A birth stool is sturdy stool designed to support upright, open positions during labour and birth. It uses gravity to encourage baby's descent, (similar to a toilet- the dilation station) opens the pelvis, and can make pushing more effective and comfortable.



A long woven scarf (called rebozo) used to gently support, rock, or lift the belly and hips. It encourages relaxation, eases tension in the back and pelvis, and can help baby move into an optimal position during labour.

Small handheld tools like birth combs or Aura Orbs can be used to provide sensory focus and pain relief during contractions. They stimulate pressure points in the hands, helping to redirect pain signals, increase focus, and create a calming sense of control during contractions.



use your brain

The BRAIN acronym is an important decision making tool that everyone should know when accessing maternity care. It can be used to make decisions regarding scans, treatment, monitoring, tests, interventions and birth plans.

Having this tool and using it alongside your own research into your options and each one can transform the way you feel about it all. Knowing exactly why something is offered, what other options there are and all about it makes it a totally different experience to feeling confused and out of control and not able to be confident in your decisions. You are in charge here and when you have all the information you need you are able to make the decisions that are best for you. You've got this!

Benefits

Find out the benefits of what's being offered, whether it's a test, scan, induction, caesarean or anything else. What is the aim here? What will it achieve? WHY is it being offered? Is it 'just in case' or is there a known reason to do it in your case specifically or is that just policy? For example an induction may be suggested due to the birthing parents age as that's what the policy is there.

Risks

What are the risks of doing what is offered? How safe is it for you? How safe is it for baby? Does it increase the chance of needing any other tests, treatments or interventions? For example, if an induction is suggested due to baby or parent being unwell, what are the risks relating to their condition, how well are they expected to cope?

Alternatives

Is there anything that could be offered instead? Instead of an induction is there monitoring that could be done? Is there another form that a test or procedure could take? Daily bloody glucose monitoring at home instead of a Glucose Tolerance Test at hospital etc. A different medication for a condition etc. When you know what all the options are then you can get informed on them all and make your choice.

Instincts

What does your instinct tell you about it? Instinct is a really important thing to consider in pregnancy, birth and parenthood. Often the parent will know that something is not ok with baby before health professionals do. Similarly, they will often feel that all is well with baby despite a professional believing otherwise. So what do you think? What is your gut feeling? Do you feel like all is well and you're happy to continue as you are or do you think what is offered might be necessary? Does it make you feel relieved to think about getting answers or baby being born asap? Or does it all feel too hasty?

Nothing

What happens if you do nothing instead? Not what's being offered, not an alternative, just nothing. What is expected to happen then? Is it nothing? Is there a small chance of something? Or is there a big chance of something occurring? It may be that you do nothing for an hour, a day a week or maybe just nothing at all as long as all is well. It's completely up to you.

Bonus one-

Know that no matter what the circumstances, you are the decision maker here. Nothing can be done without your consent and in the vast majority of cases there is time to take some time to decide, even if it's just 10 minutes. You can think this through, do your research and make your choice. If anything goes ahead it is because you've decided that it's the best thing for you and your baby! Remember a positive birth is possible. A positive birth is not a spontaneous intervention free vaginal birth or even a vaginal birth at all. Take some time to destress, practise breathing techniques, sit on it for a while if you can and then research to help you make your decision. Don't worry about what anyone else would choose, just pick what's right for you.

Lets think about some scenarios- what would you do/ask?

E..g. Internal FSE monitoring is suggested due to irregular or unclear fetal heart patterns.

B.R.A.I.N.:

- Benefits: Provides continuous, accurate monitoring for baby's safety.
- Risks: Invasive procedure, may cause discomfort or infection risk.
- Alternatives: External monitoring, frequent checks, changing maternal position.
- Intuition/Impact: Does the parent feel safe with an internal device? Will it help or hinder movement?
- Nothing/Next Steps: Discuss procedure, consent, and how it fits into labour plan.

- 1. Labour has started at home, but contractions are slow and cervical dilation isn't progressing as expected.**
- 2. Contractions are intense and exhausting. The birthing parent is considering an epidural or other pharmacological pain relief.**
- 3. Labour is underway, and the birthing parent is considering entering the birthing pool to reduce pain and encourage relaxation.**
- 4. Labour hasn't started spontaneously, and staff recommend induction using medication or a membrane sweep to help labour begin safely.**
- 5. Fetal heart patterns are unclear or irregular, and staff suggest using a fetal scalp electrode for continuous, accurate monitoring.**

pain relief options

Method & Description	Best Used When	Pros	Cons / Considerations
Breathing – Slow, rhythmic breathing to focus & relax	Anytime, especially during contractions	Easy to do, promotes calm and focus	Requires practice under intense contractions
Massage – Partner or doula massage to release tension	Early & active labour	Reduces tension, boosts oxytocin, encourages bonding	Needs someone available; may be less effective if contractions are strong
Anchoring (Visualization/Focus) – Focus on calming images, objects, or thoughts	When feeling anxious or overwhelmed	Promotes calm, reduces anxiety	May be difficult without prior practice
Aromatherapy – Essential oils for relaxation	Early labour, resting periods	Enhances relaxation and calm	Sensitivity to smells; check policies
Water (Bath/Shower) – Warm immersion or shower	Early & active labour, or pain relief	Soothes muscles, reduces pain perception	May not be available; slippery surfaces
Acupressure – Pressure on key points to relieve pain	Back or hip discomfort	Can reduce pain naturally	Needs guidance for correct points
Movement & Positioning – Walking, swaying, squatting, or birth ball	Active labour, to encourage baby's descent	Helps baby move down, eases discomfort	May be difficult if exhausted or limited mobility
Counter Pressure – Firm pressure on lower back/hips	Back labour or strong contractions	Eases back pain	Needs partner or support person
Heat / Cool Therapy – Warm packs or cold compresses	Muscle tension or swelling	Comfort, reduces pain	Requires preparation or equipment
Distraction – Music, conversation, or visualization	Mild to moderate contractions	Shifts focus, reduces anxiety	Less effective in intense labour
TENS Machine – Electrical stimulation to reduce pain signals	Early & active labour	Non-invasive, reduces pain perception	Requires equipment; positioning may be restrictive
Hypnosis / Self-Hypnosis – Guided relaxation to manage sensations	Throughout labour	Promotes calm, reduces pain perception	Needs prior learning / practice

Method & Description	Best Used When	Pros	Cons / Considerations
Entonox (Gas & Air) – Inhaled nitrous oxide for pain relief	Short bursts of contraction	Fast-acting, self-administered, wears off quickly	May cause lightheadedness, nausea, or drowsiness
Epidural – Regional anaesthesia for strong relief	Intense contractions, long labour	Strong relief, allows rest	Reduces mobility, may lower blood pressure, prolong labour
Pethidine – Short-acting opioid injection	Moderate pain	Reduces pain	Can cause drowsiness, nausea, may affect baby if given close to birth
Diamorphine – Strong opioid injection	Moderate to severe pain	Strong relief	Drowsiness, nausea; requires IV/injection
Remifentanyl – Fast-acting IV opioid	Severe pain, when epidural not possible	Rapid relief	Needs continuous monitoring; may cause drowsiness or nausea



birth positions for labour

Just because the bed is the focal point in many labour rooms doesn't mean you should head straight onto it! Being upright and active helps to shimmy that baby down into the world! Think upright, forward and open as you move around your space however feels best.

Moving in the way your body tells you to can help you stay comfortable, distracted and encourage optimal position without a single thought!

Here are some positions you can try if you can't quite work out how to get comfy!

- Use a Birth ball to sit up on; rock back and forth or in a figure of eight or u shape.
- Lean over a bed/chair and sway side to side.
- Get on all fours and rock back and forth.
- To rest, try getting on your knees and leaning forward with your arms crossed on a birth ball or chair, or exaggerated side lying.

As I'm sure you can tell, the key here is to be upright and moving for the most part.

You notice we didn't say lie down on your back like you see in the movies?



Seated Sported squat



Birthing seat/sitting backwards on toilet



Upright pelvis open

Squat



Supported squat



All fours



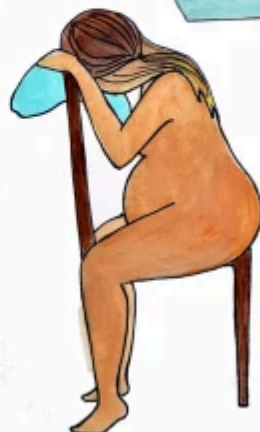
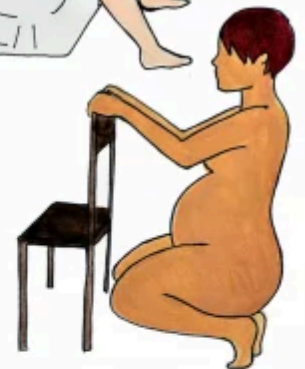
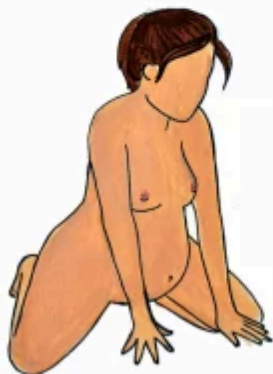
Birth pool



Use your birth ball



Labor & Birth Positions



stages of labour

The following is a general guide but all labours and births are different. Not everyone will be aware of all the stages or experience birth in the same way.

Latent Phase

This phase may start weeks, days or hours before baby is born. Signs may be feeling very emotional, mucus plug, nesting, wanting to stay close to home and changes in bowel movements. You may experience gentle surges that are regular for a while and then slow and stop again, this is very common and is all your body getting ready to birth your baby. During the time your cervix is beginning to move forwards, soften and shorten. It can be frustrating to have a long latent phase as it feels like things are beginning and then they disappear again but never forget that all of it is helping to bring your baby closer to being born. A long latent phase can mean that once the next phase starts up, everything starts to move a lot faster. Check out www.spinningbabies.com for positions and exercises to do during latent labour incase this would help things to progress. You can go about your life as usual as much as you want to during this phase.

Early Labour

By this point you are likely to be having regular surges that don't seem to be getting further apart or stopping. Changes to your environment or the people in it and stresses can still slow or stop labour at this stage though so be mindful of your environment. You may begin to think about your comfort measures such as getting in the shower or bath, massage and TENS machine. You may choose to be alone during this part in your own little bubble or have a partner, friend or family member with you. This may be helpful during the day if there are other children around or for providing food and drinks. Try to remain upright, forward and open throughout early labour but if you want to rest then do so. Care providers would measure this part as from the beginning of labour until around 4cm and a rule of thumb being until surges are around 3 or 4 in 10 minutes.

However, this is just a guide and if you begin to feel like you would very much like to get to your birth space, be seen by a midwife or receive a form of pain relief that isn't available to you alone at home, then this is probably the right time to call for a midwife to come to you or start making your way to your birth space.

Active Labour

During active labour most people will be experiencing regular surges that require more of their attention. You will likely find movement, breathing and finding a rhythm with your surges will help you to ride each one. It's common for the birther to appear a bit more spaced out or in their own zone during this part and its the birth partners role to protect that space and try and leave them uninterrupted where possible.

The birth partner can be mindful of the environment, who's in it and any conversations that are taking place around the birther. You may wish to get in a birth pool at this stage or request additional comfort measures according to your wishes.

Transition

At some point just before baby makes its way into the world, there is a surge of adrenaline through your body. The purpose of this is to wake you up out of your birthing bubble to get you ready to care for baby and to initiate the birthing phase of labour. During this time you may feel nauseous, if you need to be sick, let it out. Others around you may notice a change in your behaviour as you become more aware of your surroundings and the sensations in your body. This is the time where many birthers begin to say things like "I can't do this anymore", "I need an epidural" or "I need to go to hospital" if birthing at home. Birth partners and care providers can help the birther by providing reassurance and cheerleading. Recognising transition and knowing your baby will be here soon can help you to get through a sometimes difficult time.

The Birthing Phase

At some point after your cervix is 10cm dilated the foetal ejection reflex or 'urge to push' will kick in. The length of time between full dilation and this occurring varies from person to person. Sometimes without care provider patience, interventions can be suggested when they are not necessary. If all is well with yourself and baby then there's no need to do anything but wait. Many people call this phase the 'rest and be thankful' phase and it is great to just conserve energy and wait for that urge to kick in. Research suggests that the time between full dilation and baby being born is no different whether you wait for the foetal ejection reflex or are coached to push so save the energy and wait for your body to do it for you. As the birthing phase kicks in you may feel like you need a poo, perhaps you do! If there is poo in the way then as your baby comes down it will make its way out. Don't hold back! Your care providers will be very used to this and will be ready to clear it away respectfully. As everything in this area is so close together, the feeling of your baby about to be born can just feel a lot like needing a poo. The only way to know is to just go with it! At some point you will feel either a strong urge to bear down or that your body is doing it already and you couldn't stop it if you tried! Use your birth breathing to give you something to focus on and help it along. It's usual to make a loud involuntary 'mooing' noise at this point! Your babies head will usually be born first (unless breech) and then within the next few surges your baby's body will arrive. You've done it and your baby is here with you!

The Golden Hours

If the birther and baby are well then there is no need to interrupt the bonding process. Lots of time with baby on the birthers chest will help them both to produce oxytocin and reduce stress hormones. This aids bonding and helps with the placenta's arrival. Now is a great time for baby to have their first feed if you are planning to breastfeed, feeding soon after birth is associated with easier breastfeeding journeys. Things like weighing or measuring baby and getting baby dressed can wait until the birther is ready to have someone else hold baby while they get dressed perhaps.

Clamping and cutting the cord

'Delayed' or optimal cord clamping should now be standard across all UK hospitals. At the time of baby's birth there is still up to a third of baby's blood inside the placenta, waiting to cut the cord until after this blood has been returned will improve baby's iron levels. Once the blood has left the placenta the cord will have gone from purple to white. The length of time this takes varies from person to person. You may wish to clamp (or use a cord tie) and cut the cord once this has happened or wait a while later, it's up to you!

Birth the placenta

After the birth of baby you are still technically in labour and so oxytocin and movement still help things along. Lots of people find sitting on the toilet helps to birth their placenta or just getting up and moving a bit. As previously mentioned skin to skin with baby can help boost oxytocin and cause the uterus to have the surges needed to detach and birth the placenta. You may be offered an injection of synthetic oxytocin to speed this process along. You can choose to have this or wait, there are pros and cons to both. Look into your options and decide which one you would like to go with. You can always opt to wait for the placenta to be born naturally and change your mind at any time. You're in charge here!

Latent Phase	1 st Stage of Labour	2 nd Stage of Labour	3 rd Stage of Labour
Waters breaking?	Waters breaking?	Waters breaking?	Baby skin-to-skin
Mucousy/bloody Show?	Mucousy/bloody Show?	Mucousy/bloody 'show?	Baby feeding at breast
Backache	Has to breathe and focus through surges	Has to breathe and focus through surges	Some further contractions felt
Cervix long and closed	Cervix soft and effaced	Cervix open 10cm	A slight gush of blood
Cervix soft and effaced	Cervix open 4cm	Rectal Pressure	Placenta birthed
Cervix open 1-2cm	Surges strong, 3 in 10 minutes, lasting a minute	Involuntary pushing	
Period-type pains	Time to call midwife	Opens bowels	
Can talk through surges		Vertex (top of head) visible	
Surges moderate, 2 in 10 minutes, lasting 40 seconds		Crowning	
		Head birthed	
		Baby born	

birth planning

Firstly, writing a birth plan is NOT a silly idea or an invitation to be disappointed. Anyone who thinks that way is not yet informed about how bloody important a birth plan is! A birth plan is not a prescription or a set of instructions, its just a clear presentation of what you want! It will help your care providers to know exactly what you want without asking you a million questions but most importantly it will help YOU and your birth partner/s to learn all the options available and know what you want!

Visual Birth Plan Icons. You can find these on www.pinterandmartin.com. These are a great way to have a first look at what all the options are. There are likely to be lots of them that you hadn't considered or have just never heard of before. Find out what they all mean if you're not sure, knowing what all the options are is a great way to start figuring out what you would like to choose!

You are looking to create AT LEAST 3 birth plans. Think about what changes could happen. Eg. Home birth to hospital, spontaneous to induction etc. You would then create birth plans for each scenario with plan A being your ideal birth and plan C being a caesarean birth. Someone planning a Labour ward birth may do a plan A for that ideal birth they are work- ing towards, a plan B for if they decide to have an induction or any augmentation (waters being broken, the drip etc.) and then a final plan C for caesarean. If plan A was a home birth or birth centre birth then there may be a plan B x2 for a change of birth place too!

Think about what you want! What's important to you? What birth have you always thought would be lovely? At this point it doesn't matter what type of birth you think you are limited to, for now you are just getting an idea of the type of birth you would love to have. The next steps will help you find out how you can make this happen or what alternatives there are.

What special circumstances do you need to consider? Find out everything you can about these- Places like Evidence Based Birth, Sara Wickham's website, the NICE guidelines, AIMS. These may be health issues of your own, pregnancy related issues, health issues for baby. Not all of these will affect your birth choices but now is the time to find out which ones do if you have any!

Find out others experiences. For example, If you would love a home birth and you have gestational diabetes, Join a FB group like Home Birth Support Group UK and look for others experiences with GD. This can be a great way of getting some insight into what other people have found useful when making their decisions and the different care provider opinions others have come across. This might show you that it's not as black and white as you may have been led to believe. No matter what it is always your choice.

Talk through your options with knowledgeable folks. Your instructor or the AIMS helpline would be a good start, they can tell you what they know, any relevant research and help you come up with a list of questions for you to ask your care providers as well as stuff to read up on.

Discuss with your care provider. This may be a midwife or a consultant, do not be disheartened if you don't hear what you would like to hear, you can still chat through your circumstances with a Professional Midwifery Advocate and they can talk you through if what you are after is REALLY out of the question or not.

Consider what things are super important to you and you would not like to miss out on unless there's an emergency. This may be remaining upright throughout labour and for the birth, skin to skin after birth, avoiding 'the drip' or any other intervention or medication etc. How can you try to make sure that you can have these things? Can they be worked into all your birth plans? Eg. Remaining upright- even in the case of epidural use you can be assisted into various positions that support birth. Skin to skin after birth, can it be arranged that your birth partner holds baby on your chest for skin to skin EVEN after a Caesarean under general anaesthetic? Avoiding 'the drip', this may mean you opt for a caesarean if you would rather. There are so many possibilities we're not always offered, if you aren't sure how you can still incorporate certain things then talk to your instructor!

Create the birth plans for real! Consider everything you've learned about any factors concerning your pregnancy and chats with PMA or midwife etc and bring it all together to create your plan using whichever format suits you best, try to ensure it will be easily read by the care providers and explained by your birth partner.

Create your plan methodically by first considering your background such as how you feel about birth, how many babies you've had, any special medical or cultural requirements.

Then move onto the things that you would like to include in your environment such as low light levels, quiet, who you would like present (eg. Are you happy to have student midwives present? A doula?).

Next consider what you would like for each stage of labour, comfort measures you'd like to have available, what monitoring or interventions you would like to choose or avoid. Make sure to include what you would like your midwives to do, whether you would prefer them in the background or if you would really like the reassurance of having them right next to you when they're in the room. Include how you would like baby to be born, whether you would like direction from a caregiver or not and if you would be happy to accept forceps etc if thought necessary.

Then consider what you would like to happen once baby is here, would you like to find out the sex yourself?

Who will cut the cord? When will it be clamped (if at all)?

What about vitamin K injection for baby? Let those reading know how you would like to feed your baby and what you would like to happen with baby when they are born, would you like uninterrupted skin to skin?

Would you like baby wiped down before you hold them? This is all up to you! Finally, how would you like to birth the placenta? What would you like to do with it after?

Do this with your birth partner ideally so that they understand it all and know the reasons for your choices. At the very least, go through it in detail with them, they will then be able to help relay it to your care providers on the day.

Change your birth plan any time you want, we often learn new things and make new choices. Finally, you can change your mind on the day if you want to!

Download the birth planning guide to access your free templates for both vaginal and caesarean births.

www.birthing-hood.co.uk/clienthub using the password sent to you!

You'll also find a range of other resources to support your pregnancy and birth journeys like this birth partner quick guide!!

Birth Partner Quick Guide

The Big Picture

Position changes can help baby rotate, descend, and align
Think movement + asymmetry (one knee up, one down)
Back pain often means baby needs help turning, not just pushing harder
Change position before assuming something is wrong

Early Labour / Labour Slow to Start

If contractions are irregular or fading: Stay upright: walking, swaying, leaning forward
Try Forward-Leaning Inversion (with support)
Use Side-Lying Release (both sides)
Avoid lying flat on the back
Partner role: Offer steady support
Time positions (10 minutes or 3 contractions)
Reassure: "We're helping baby find space."



Active Labour / Labour Stalls

If contractions weaken or space out: Change position every 20-40 minutes
Try:
• Lunges (opens mid-pelvis)
• Hands and knees Leaning over a bed, chair, or ball
If baby may need help rotating:
• Shake the Apple Tree (gentle hip jiggling)
• Sifting with a scarf or rebozo
• Use asymmetry: one knee open, one closed



Knees: When to Open vs When to Narrow

Open Knees = More Space (Descent)
Use when:

- Baby feels high
- Pressure without progress
- Before pushing

Good positions:

- Supported squat
- Toilet sitting
- Birth stool
- All fours with wide knees
- Birth ball open knees

Narrow or Asymmetrical Knees = Rotation
Use when:

- Strong back pain
- Baby feels stuck
- Labour slows despite strong contractions

- Good positions:
- Side-lying with top leg supported
 - Hands and knees (knees under hips)
 - Lunges

Birth Partner Quick Guide

Back Labour & Intense Pelvic Pressure

Use Counter Pressure

When:

- Pain is in the lower back
- Birthing person asks for pressure
- Contractions feel overwhelming

Options:

- Sacral pressure (flat palm on sacrum whenever needed)
- Hip squeeze (during contractions)

Partner tips:

- Use body weight, not just hands
- Pressure should be firm and steady
- Stop if it increases pain



Pushing Stage

If pushing feels ineffective:

Change position:

- Side-lying
- All fours
- One leg up/one leg down
- Avoid long periods on the back

Partner role:

- Support legs or hips
- Help maintain balance
- Encourage rest between pushes

Partner Quick Check

- Have we changed position recently?
- Are we upright or forward-leaning?
- Do we need more space or more rotation?
- Would gentle pressure help right now?
- Have they been to the toilet recently?
- Do they need hydrating or fuel?



You don't need to fix birth, just help create space, comfort, and confidence

Leanne xx





birth proposal

NOTES TO MIDWIFE

- Name:
- Birth partner's name:
- Homebirth
- Doula: Leanne
- Gender unknown

ENVIRONMENT

- Low lighting
- Aromatherapy
- Galaxy lighting
- Birthy playlist
- TV shows
- Birth ball
- Eye mask
- No mention of time
- Low communication
- Birth pool

PAIN RELIEF OPTIONS

DO NOT OFFER PAIN RELIEF

- Breathing
- Water
- Comb/Acupressure
- Massage/counter pressure
- Encouragement/Affirmations
- Cold Flannel/fan
- Essential oils
- Birth ball
- TENS Machine
- Gas & Air
- I am open to other medication if transferred and I request

MONITORING

- Intermittent monitoring
- I do not want vaginal examinations
- Check waters and signs of infections via temperature and baby movements if SROM/PROM
- No offer of induction on due date
- After 41 weeks extra monitoring and open to discussion for next steps at 42w

EXAMPLE

COACHING

- Please encourage 'breathing baby down'
- Go with body (let go)
- Encouragement and 'you're safe'
- I do not mind being coached to push if absolutely necessary. I will communicate if my needs change and therefore no longer require support.
- I do not want to push in between contractions.

BIRTH POSITIONS

- Birth in water
- UFO positions - I want to move freely
- (Over ball, side of pool, lunge)
- Leanne to assist if necessary

EDD:

First/subsequent pregnancy/birth



birth proposal

INTERVENTION

- No intervention (unless absolutely medically necessary and discussed with Leanne)
- No sweeps
- No cannula just in case
- I do not want assisted delivery unless absolutely no other choice.
- I do not consent to an episiotomy.

CATCHING THE BABY

- I would like help to catch baby and bring to chest, **to help and confirm gender of baby.
- If I birth in water I would like to remain in the water until I specify.
- Hands off approach.

3RD STAGE

- Natural expulsion of placenta (no injection unless signs of PPH or maternal request)
- WAIT FOR WHITE – Optimal Cord Clamping (potential lotus)
- *** to cut cord
- Placenta to be encapsulated
- Placenta checked in view- We would like to see it.
- Using a cord tie

GOLDEN HOUR

- No hat when skin to skin unless baby in compromised
- Skin to skin (at least 1 hour)
- ** skin to skin after
- Delay weighing baby (at least one hour)
- Breastfeeding
- Vitamin K via injection
- I consent to NIPE check

EXAMPLE

NOTES

I wish for my birth partner to confirm the gender of the baby to me.

Please have any discussions with my birth partner/doula in the first instance, for them to relay the information to me.

Please avoid any discussions about me in the room.

Please could these be conducted elsewhere.

Please allow me to move freely and use the toilet facilities as required.

caesarean birth

Hopefully this will help you to become more familiar with the process of caesarean. Caesareans obviously happen for various reasons and it is not always known that that is going to be the mode of birth, in this case, the pre op appointment below would not happen and the preparation on the day may vary a little but many of the info is relevant to various scenarios.

Pre Op

If a caesarean is planned, either booked for a certain date or for when labour begins, there'll be a pre op appointment beforehand in order to make sure the birther and the medical team are as prepared as possible. At this appointment there is a usual antenatal check, listening in to baby, blood pressure, urine dipstick etc. There is a blood test for iron levels and there is talk about anaesthetic, the birther may be weighed if they are happy to be.

On the day of an elective caesarean

On the day of a planned caesarean, the birther will be asked to fast. The length of time will vary based on the local policy. The birther will be asked to thoroughly wash their body and hair with a disinfectant wash prior to coming in. They will go into the hospital first thing in the morning. Once there they will have another antenatal check and be given a gown and surgical socks to put on. The birther will be given an antacid to prevent aspiration during surgery. There is likely to be a tremendous wait depending on who is in need of the theatre and team that day. It's a great idea for people to expect a long wait so that they will not be disappointed. The order will always be based on who is in most need and what is safest for everyone. Usually elective caesareans are less urgent than those who have been in labour and now require a caesarean.

Relevant to most caesareans

Once ready, the birther will be greeted by the team who will be caring for them that day. This will consist of 2 surgeons, a scrub nurse, anaesthetist and an operating department practitioner (assistant to the anaesthetist). The birther will receive their anaesthetic and the anaesthetist will remain by the birthers head throughout checking to see that all is well with them. The surgeon will test the site to check that it is numb and if so, they will proceed with the caesarean. The baby will be born within the first 10 minutes and the whole process will take about an hour including removing the placenta and stitching everything back together. After this the birther and their partner will be moved to a recovery room nearby for observations before being moved to the postnatal ward.

Options

There are many different options when it comes to caesareans despite what is commonly thought. There are lots of different things that can be chosen in order for each birther to have a caesarean that they are happy with in most cases.

- If there is no urgent need for baby to be born then there is the option for baby to be born slowly and gently to mimic vaginal birth in that the baby is squeezed by the incision and this can help to clear the baby's airways of fluid.
- The birther may wish to have their ECG dots on their back so that their chest is clear for skin to skin after the birth.
- They can choose which hand their IV goes into so that it is not in the way as much.
- They can have the screen lowered to be able to watch baby enter the world, due to the bump and the angle of the birther they will be unable to see the incision but can see the baby as they are born.
- They can opt to have the surgeon talk them through what's happening at each point, some people find it comforting to know and others don't want to know at all.
- You can request optimal cord clamping which should now be standard after all births where possible but it is always a good idea to add it to a birth plan.
- Skin to skin should be standard and birth partners can help to facilitate this if the birther finds it difficult to do this for themselves.

Ways to remain calm in the lead up to a caesarean

Whether you have weeks, days, hours or less to prepare for your caesarean, there are various things you can do that may help you to feel calm.

- If you would like, you could watch positive caesarean videos, concentrate on the face of the birther as they meet their baby. Sometimes it's the unknown that is scary so just seeing what the process is can make it seem less so.
- Listening to MP3s and using breathing techniques whilst travelling to the hospital and during any periods of waiting can really help to keep the oxytocin flowing and the adrenaline at bay, making it feel much more positive.
- Making sure expectations are managed can help prevent panic on the day, it is likely that there will be a big wait before a caesarean and this could even mean that the date the caesarean takes place is not the date it was booked for, this can be really disappointing or shocking if unexpected.

Post caesarean tips

- Pressing a clean muslin, sanitary pad etc against the scar whilst coughing, sneezing or having a poo can help it to feel more protected.
- Chewing gum or drinking mint tea can help with any trapped wind which can make for a lot of discomfort after a caesarean.
- It's advised to keep on top of pain relief in the first few days even if you feel comfortable when pain relief is due. Once the pain relief wears off it is harder to get on top of the discomfort and so it will be more comfortable to take pain relief at the suggested intervals just in case to start with.

stretch & sweep

A sweep is a form of induction as it aims to encourage the body to go into labour before it would have happened on its own. However, people are not encouraged to be in hospital or anything after having one so if labour begins after that you would go on to labour in your chosen birthplace etc. They are often offered routinely at the end of pregnancy, the gestation that this would happen will vary based on area.

Process

Via a vaginal exam the midwife or doctor will insert a finger into the cervical opening, if possible, and try to run a finger around the inside to separate the membranes (the sac containing your baby) from the wall of the uterus. If the cervix is not open they can sweep the outside. A sweep aims to irritate the cervix and uterus which releases prostaglandins. Usually a certain amount of discomfort is experienced during a sweep as it is not a gentle procedure.

Evidence- It's impossible to get accurate evidence on sweeps as if someone goes into labour the day after a sweep, it is impossible to know if they would have gone into labour then anyway, since sweeps are only given at the end of pregnancy when labour is likely to happen. The evidence from a Cochrane review that brought lots of studies together showed that the use of sweeps in first time parents did 'not seem to produce clinically important benefits' when used to prevent the need for further induction interventions. However many birthers would like to try it as a first step to see if they can avoid induction. Sometimes this can have positive mental benefits as it can make you feel like you've tried all you can.

It is worth remembering at this point that the cervix can change quickly so if you attempt a sweep and find that your cervix doesn't seem to be gearing up for labour then don't be disheartened as you could still go into labour any minute! Get that oxytocin flowing!

Side effects

- Sweeps can induce discomfort similar to surges/prolonged early labour which can lead to exhaustion and is linked to higher epidural use and further interventions.
- They can cause bleeding and discomfort to the cervix which can impact oxytocin even if it turns out to be harmless.
- In some cases the membranes (your waters) can be broken and this would then begin the clock on your local policy of how long to leave labour to begin before stepping in to induce or offer caesarean. It is always up to you how long you would like to leave it or if you are happy to go with the recommendation but it can be more decisions to make and can impact oxytocin.

Resources

What is a stretch and sweep? Sara Wickham

EBB Podcast- Episode 6- Pros and cons of membrane sweeping

induction

NICE guidelines state that induction has a large impact on the birth experience, it is more likely to result in an epidural and assisted delivery. It is a big decision and always up to the birthing person.

Alternatives could be waiting or offering increased antenatal monitoring including scans, depending on the reason induction is suggested. It is also your right to request an elective caesarean if you would rather go down that route rather than attempt an induction.

It should be clinically justified, there should be a reason that it is safer for baby to be born than remain in the womb and that induction is the way the birthing person would like to do that.

1 in 3 labours are induced, 70% for dates alone. It is important to get informed about induction as there is a good chance you will be offered one at some point in your pregnancy.

'The pessary' is a synthetic prostaglandin inserted next to the cervix to soften it ready for labour. In some trusts this is offered as an outpatients procedure but in most areas this is something that you have done in hospital on a ward. Depending on the method of this, this may be left in for 6 hours or 24 hours and there can be multiple tries. This part of induction can be very long so make sure you are prepared for a wait and have brought entertainment with you.

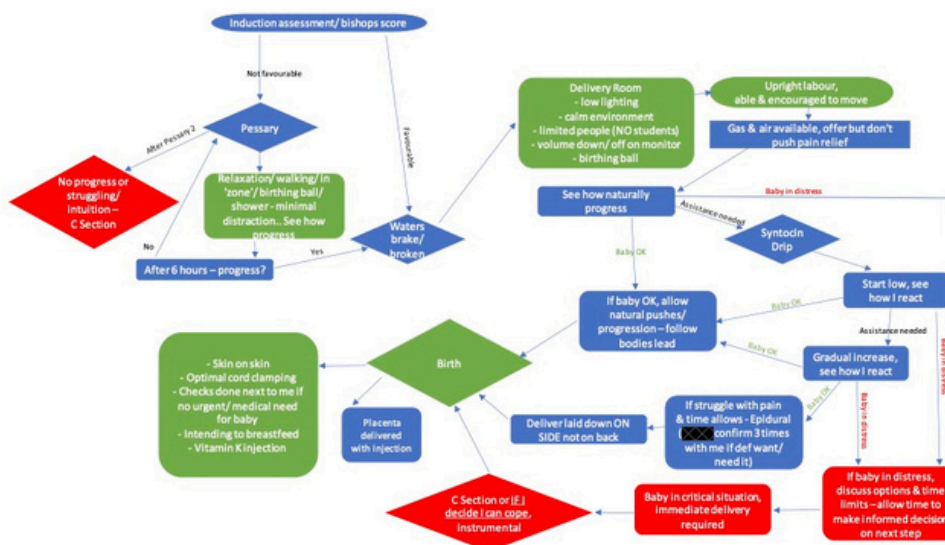
Water can be broken if the cervix is open and the birther consents to this. This is done using a knitting needle type tool, the vaginal exam is the only part of this which may be uncomfortable. The aim of this step is to have baby's head make contact with the cervix and help dilation along. Labour can become more uncomfortable after the cushioning of the waters has been removed so have a think about this in your birth plan. Include how you would like to manage your comfort levels.

If no contractions start then the drip will be offered, syntocinon is an artificial oxytocin which will cause contractions. Unfortunately not the same as natural oxytocin in that it doesn't make you feel happy and full of love as it doesn't enter your brain, it affects only the uterus. Monitoring of baby regularly or continuously is often suggested when using the drip to keep an eye on how baby is coping with the surges. Contractions from syntocinon are said to be very intense and have no real breaks in between them as spontaneous surges would. This is very normal and not a reflection of how well you can cope with labour. Don't be disheartened if you would like stronger pain relief for this.

The drip and potential continuous monitoring (if wanted) can hinder the ability to move freely which then can prevent optimal positioning of the baby and for baby to make the little adjustments to aid birth along. If hooked up to any wires, there are lots of ways be upright and active without laying on a bed. Standing and leaning over the bed, getting on all fours on the floor (often hospital staff will even assist you to put the bed mattress on the floor for this, talk to them about your wishes!

Tips for a positive induction

- Read positive induction stories.
- Always think of your environment and supporting your hormones.
- Remain upright and active where possible.
- Make sure your birth plan covers all possibilities so that you feel calm and prepared.
- Ensure your birth partner knows all your wishes and is prepared to manage the space and anyone in it.
- Communicate with your care providers about what you want beforehand.
- Manage your expectations so that you are ready for a potential long wait and understand each step of the process.



Some great resources for induction are below, the more you know, the better you do!

- Sara Wickham- '10 things I wish every woman knew about induction of labour'
- NICE Guidelines- Inducing Labour
- FB Group- Positive Induction Birth
- The Minimalist Doula- Doula's Guide to induction
- Book- Inducing Labour by Sara Wickham
- Book- Why Induction Matters by Rachel Reed

post dates

70% of inductions in the UK are for post dates. Research into due dates etc tells us that most people will go around 10 days over with their first baby. Only around 50% of people will have had their first baby by 40+5. The remaining 50% will be sometime after that. This shows us that it is actually pretty unlikely to have a baby around your due date really! Only 4% of babies are born on their due date. Term is anytime between 37 and 42 weeks and so a baby is not actually 'late' until 42+1.

Policies on when post dates inductions are recommended vary across the UK and the world. In the UK policy usually dictates that induction is recommended sometime between 40+5 and 40+14. It is always up to the birthing person to decide whether this is something they would like to go ahead with or if they would like to wait.

Options when reaching the end of term

Simply waiting

- Just declining induction and waiting for your baby to be ready to be born is fairly self explanatory of course! If you feel that you would rather wait and go into labour spontaneously to avoid the risks associated with induction then this is totally your choice and with the risk of stillbirth being so very low it is absolutely not ok for anyone to try and use them to influence your decision and make you feel bad. After 42 weeks anyone can have a home birth or use the labour ward but you may need to negotiate consent to use a midwife led unit past your trust's cut off as this is NHS property.

Going for regular monitoring

- This will very much depend on how often you would like to go and do that. You may be offered the chance to go in daily for various monitoring but it is up to you if this is what you are happy with or if you would rather it was less frequent. Some may find the monitoring reassuring and others may find the trips to hospital to be stressful. Other trusts may suggest monitoring less frequently than every day.

There are different forms of monitoring that may be offered. There is continuous cardiotocography (CTG monitoring), this is when the elastic straps are placed round the bump and monitor baby's heart rate and uterine contractions. An ultrasound scan may also be offered to check the flow of blood through the umbilical cord, check that the placenta appears healthy and amniotic fluid levels. Babies growth will also be examined during these scans but do bear in mind that these scan be very inaccurate, babies rate of growth (ie. are they getting everything they need to keep growing steadily) is more important than their estimated weight.

Having an induction

- Currently the NICE guidelines state that induction should be offered between 41 and 42 weeks of pregnancy. Many people are led to believe that this is because the chance of stillbirth after this time is high. However, the research into this has shown us that if the chance of stillbirth increases at all (studies show various different results) then the chance is small at any point of pregnancy. Less than 2 babies per 1000 births.

A recent Cochrane review estimated that 426 people would have to be induced between 41 and 42 weeks of pregnancy to avoid one stillbirth. Now if you are happy to go ahead with induction at the recommended time for your trust, then this is absolutely fine. If you would like to pick a point of pregnancy that you would be happy to get to before accepting an induction then this is your choice, so if your trust suggests induction earlier than your neighbouring trust but you would like to go with the later date then discuss this with your midwife. You're in charge here and you can have a far more positive induction experience when it happens on your terms.

Having a caesarean

- If you would like baby to be born by a certain number of days past your due date but are not happy to consent to an induction then you may request a planned caesarean. The date that this happens may depend on the hospitals schedule and also what urgent cases they have on the day. Some care providers may be more in support of this than others so take information about your decision along to any appointments so that you can explain that you are informed and have a two way conversation about your circumstances.

No matter what you decide to do:

Think Upright Forward and Open, UFO

- Babies positioning can sometimes be what is delaying labour or slowing progress. Staying in positions that encourage optimal positioning (head down, back to birther's front) can help baby to make the necessary adjustments to start making it's way out! Sitting on a birthing ball, standing and swaying or getting on all fours can help with this. If you want to rest then kneeling on the floor and resting your head on your crossed arms on top of a birthing ball or sofa may be what you need! Check out www.spinningbabies.com for positions and exercises to try. Oxytocin is key.

- With spontaneous labour, induction or caesarean oxytocin will always help it along or help you feel good during! Making sure you're feeling as relaxed and happy as possible will help you in whatever form of birth you decide on. This is of course easier said than done as late pregnancy is a very emotional time for everyone. Spend time with people close to you, do things you enjoy, eat nice food and watch your favourite movies and TV. Do what you can to get some sleep or rest using your relaxation MP3s, relaxing smells, a bath or whatever it is you like to do to relax! You have totally got this, these are the last hours or days before your baby arrives, this won't last forever!

Other alternatives

- There are many other things that people swear by for late pregnancy encouraging baby to arrive, there is limited research into a lot of things (which doesn't mean they can't be effective) so read into anything you are considering and consult your midwife. Here are some commonly recommended ways of encouraging babies arrival gently- dates, acupuncture, reflexology, visiting a chiropractor, evening primrose oil, clary sage oil, dancing, nipple stimulation and orgasms (you probably don't need to ask your midwife if those are ok! Just be mindful not to pop anything in your vagina once your waters have released).

Resources

- Evidence Based Birth- Evidence on: Inducing for Due Date
- Sophie Messenger- The Myth of the Ageing Placenta
- Book- Inducing Labour by Sara Wickham
- FB Group- 10 Month Mamas

Notes:



breech babies

By 36–37 weeks of pregnancy, most babies will be head down. Some babies may be bottom down or 'breech'. Breech babies are a variation of the norm, they are just a lot less common than head down babies but this is not usually a problem. If baby is not head down by 37 weeks there is still time for baby to turn before labour begins. Only 3–4% of babies are breech at the time of delivery. People used to deliver breech babies vaginally all the time many years ago. At some point caesareans began to be recommended and over time as this became common, care providers lost confidence in how to support breech birth as many had never been in attendance at a breech vaginal delivery. Research shows that having a confident and experienced care provider in attendance is proven to have a huge effect on the safety of breech vaginal delivery.

Options with a breech baby

ECV– External Cephalic Version

ECV is performed by a healthcare professional applying firm pressure to the outside of the bump to encourage baby to turn head down. Often a muscle relaxant is given via injection to relax the stomach and give baby more room to move during the procedure, ECV is more successful with this method. Before ECV there is usually an ultrasound to check that baby is breech and then another one after the procedure to check if baby is head down. The pulse and BP of the parent is checked before beginning. The baby's heart rate will be monitored during and after the procedure and there is a 1 in 200 chance that an emergency caesarean may be required after ECV due to changes to baby's heart rate or bleeding from the placenta. Overall ECV is successful in 35–57% first pregnancies and 52–84% of subsequent pregnancies. Read up on the procedure before deciding whether to go ahead with it.

Planned Caesarean

Some people may choose to book a caesarean to birth their breech baby. This can be booked for a certain date, an elective caesarean or just planned for when labour begins, a planned caesarean. Some people prefer to have some idea of when their baby will be born and others like to know that a) their baby was not going to turn head down on it's own before labour and b) that their baby is born when they are ready. It is up to the birther to decide what option they are most comfortable with and discuss it with their care provider.

Vaginal breech birth

Many people will choose to plan a vaginal breech birth. If this is their preference they should discuss their specific circumstances with their care provider. They should take into consideration the position of the placenta, the presentation of baby and any special circumstances. The risks associated with a breech vaginal delivery are a small chance of baby's head or shoulders becoming stuck on a part of the parent's pelvis and requiring some maneuvers to help them out. There is also a chance of the cord coming out first, cord prolapse. In the case of cord prolapse an emergency caesarean would be recommended to prevent the cord being trapped by baby's body and reducing the oxygen to baby. If the birther is informed about their options and the pros and cons of each then they should choose whatever method of birth they feel is best for them. The birther should ask about their care providers experience with breech vaginal birth to ensure that those in attendance will be confident on the day. Breech vaginal birth is often most successful when the birther is in a hands and knees position and the care providers remain as hands off as possible. If planning a vaginal breech birth then get informed about breech birth, read positive stories and ensure you create a breech specific birth plan to help you smash it! Don't forget to do a head down birth plan too incase baby decides to flip before the big day!

Other things to try

If baby is in a breech position then www.spinningbabies.com will have positions and exercises to help encourage baby to turn, it's worth doing these to encourage a head down position in the third trimester. Other things that people try to turn babies are moxibustion, acupuncture, visiting a chiropractor and hypnosis. Look into each of these things before undertaking them and discuss with your midwife if unsure if recommended for your circumstances.

Resources

- RCOG- Breech Baby Information Leaflet
- Sara Wickham- New Research on Upright Breech Birth
- FB group- Breech Birth UK
- Podcast- Birth Kweens Episode 72: Breech Birth with Nicole Morales



big babies

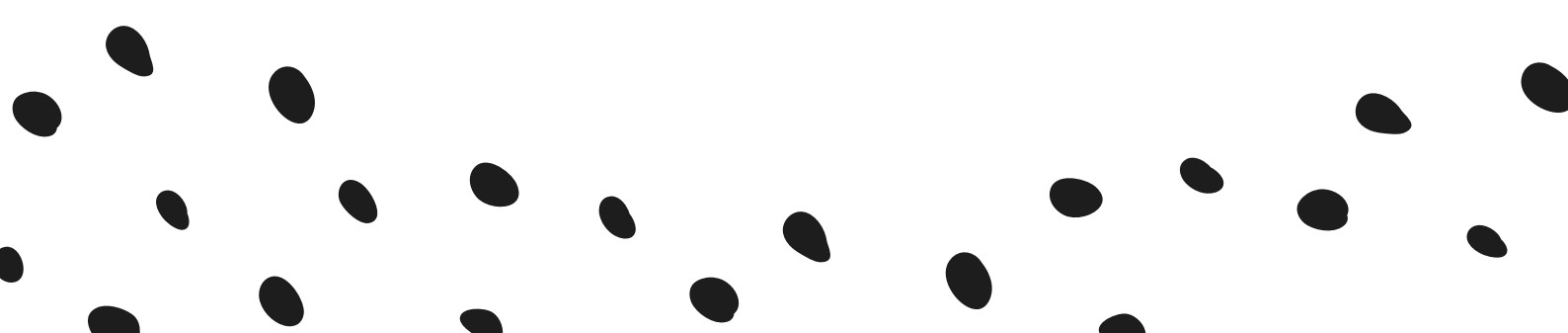
One of the common things that parents and care providers seem to worry about are babies that are estimated to be 'big'. In the UK we class babies larger than 8lbs 13oz to be big or macrosomic. However, The only way to accurately measure how big a baby is is to weigh them after they have been born.

Scans to measure the size of baby can be very inaccurate and the weight estimation can be out by 15–20%. A baby that is estimated to be 8lbs can be anything from 6lbs 13oz to 9lbs 3oz.

If used regularly throughout pregnancy then they are more accurate for plotting the growth of a baby but as a one off near the end of pregnancy they don't tell us much. As many care providers will still suggest induction or caesarean for a suspected big baby, despite it going against the NICE guidance, growth scans in late pregnancy are associated with a higher chance of induction or caesarean. If you would like to accept a growth scan that is recommended to you during pregnancy you are able to decline any recommendations given based on your baby being estimated large. You can also choose to go ahead with any recommendations given if that is what you prefer or decline the growth scan altogether. You may also wish to ask for another scan in a few days time etc. in order to see if the two measurements are the same before making your decision.

The NICE guidelines state- 1.2.10.1 In the absence of any other indications, induction of labour should not be carried out simply because a healthcare professional suspects a baby is large for gestational age (macrosomic).

In a study on size estimations in the US, 1 in 3 of the women in the study were told their baby was 'too big'. The average weight at birth of the babies in the 'too big' category was actually 7lbs 13oz. Another US study looked at the outcomes of people who were estimated to have a big baby and compared them to the outcomes of people who had a big baby unexpectedly. The study showed that the first group were 3x more likely to have an induction or caesarean and 4x more likely to have severe perineal tearing and postpartum hemorrhage. This shows us that some of the things we worry about occurring due to big babies are not caused by big babies themselves. Instead they may be caused by the management usually recommended when they are suspected to be big or the confidence issues that worrying about issues caused by having a big baby can cause.



One of the things that is talked about often when it comes to having a bigger than average baby is baby becoming 'stuck' as it is born or Shoulder Dystocia. Look into the statistics around shoulder dystocia and you will see that although spoken about often, it is still a small percentage of babies who experience this and that it affects small babies as well, just larger babies slightly more often. Shoulder dystocia rarely leads to any adverse effects to baby and is more likely to require a simple change in position as baby comes out to give baby room to move out. In order to prevent litigation against them, some care providers like to be very clear of the slight increased risk to larger babies in order to insure parents are informed of the possibility, but this can feel quite scary. Seeking out the evidence on shoulder dystocia can be really reassuring.

Resources

- NICE Guidelines- Inducing Labour
- Sarah Ockwell-Smith- The Curse of (Mis) Diagnosing a Macrosomic Infant
- Evidence Based Birth- Evidence on: Induction of Caesarean for a Big Baby.
- Book- Why Induction Matters by Rachel Reed
- Podcast- Birth Kweens Episode 110: Inductions & Caesareans for Suspected Big Babies with Rebecca Dekker

Notes:



homebirth

Why home birth? (If you've decided to have a home birth then it's handy to know this sort of info for conversations with friends and family if you wish to tell them your plans (many prefer not to if they don't expect support)).

Increased oxytocin being in a place that feels safe and private means:

- A shorter labour
- Feeling more relaxed
- More comfortable labour
- Less chance of interventions
- Less likely to have a post partum haemorrhage
- Increased chance of VBAC (Vaginal birth after caesarean)

As well as that there are all these pros:

- Less chance of getting an infection
- More comfortable for birth partner
- No limit to number of birth partners
- You can guarantee access to a pool via renting
- No need to sort childcare/ petcare

It's generally very safe! Studies show that home birth is as safe as hospital for first time birthers and safer than hospital for second time birthers. There is thought to be an increase in poorer outcomes for baby but we must remember that this includes lower apgar scores at birth (but actually baby is totally healthy).

Equipment you may need

- Pool (if you want one)
- Towels
- Some shower curtains/ tarpaulin
- TENs machine
- Comb (for acupressure)
- Aromatherapy
- Candles
- Fairy lights
- Affirmations
- Clothes for baby
- Flannels
- Snacks
- Entertainment



Things to consider:

Distance

The distance to the nearest hospital may be a factor for or against home birth, some people home birth to avoid a long journey to hospital and others opt against home birth as they have concerns about the distance if they were to transfer in an emergency. It's worth remembering that if you and baby were taken in by ambulance during an emergency the transfer would be faster than it would normally be in a car. Transfers to hospital from home births are much less likely to be because of an emergency though, the most common reason for transfer is at the request of the birther. This may be because they would like pain relief that is not available at home, they would like their labour to be augmented in some way or they just have a feeling they would like to be at hospital.

Space

Do you have an area that you can use to birth? You can give birth in any size house, room etc. Many babies are born in small bathrooms and bedrooms. Basically if there's space to live there then there is space to birth there! Some people who live in shared houses for example and can't guarantee privacy and would rather go to an MLU instead.

Childcare

Many people wish to have their children around them during their home birth and so don't have to arrange having children picked up etc. however they may still wish to have someone extra at hand to entertain the children if needs be. Others would like to have their children somewhere else and not have them there as a distraction, usually this just means having someone ready to pop by and get the children when they wake in the morning rather than getting them anywhere in the middle of the night so this is a common reason for having a home birth.

Mess

One of the things people really worry about is the mess they will make at home. Home birth is really not as messy as you might think but it helps to be prepared with shower curtains, towels and blankets you don't mind getting a bit messy, it will probably wash off! The midwives will have incontinence sheets with them which they will put down to protect things if the placenta is delivered on the sofa for example. Lots of people layer up their bed sheets with mattress protector, sheet, mattress protector, sheet and then just strip off the top two layers to get rid of any mess before getting into a nice clean bed! The midwives will pack away all their incontinence sheet and help gather up things like messy towels etc. You'll be surprised how quickly it looks like nothing happened!

Comfort Measures

At home there are not the same options of pain relief as more clinical settings and although these methods are rarely necessary due to increased oxytocin and endorphins, it can be useful to have other comfort measures at hand. A TENS machine, massage, aromatherapy (ensure pregnancy and baby safe) and water are all great relief. A birth pool is a lovely relaxing way to keep comfortable and can be great for keeping everything contained too! They are easily purchased or rented and easy to set up and pack away again. A simple shower can be a lovely way to keep comfortable as the sensation on your skin is a lovely soothing distraction.

Everyone is allowed a home birth

Many high risk people are still very safe to birth at home. Blanket policy does not fit everyone and policies can vary across the country. It is best to become informed and make your own decisions using the BRAIN tool and lots of research. If home birth is not safe for you and your baby then you will come to that conclusion and make the decision yourself. If you research your circumstances and feel that home birth is the best option for your family despite that being against your trusts recommendations then that is your right! You don't have to wait to have your home birth okayed by anyone if that's what you've decided on!

Resources

- Why Not Home? Documentary
- Why Home Birth Matters- book by Natalie Meddings
- Birth Kweens Podcast Episodes- All About Home Birth Parts 1-3
(episodes 103-105) The Birthplace Cohort Study

Notes:



birth bag checklist

For you:

- Your notes
- Your birth plans
- Your phone or camera whatever you wanna take pics on
- Your relaxation MP3s where your birth partner can find them
- Headphones for staying in your bubble
- Relaxing or uplifting music on a playlist
- Affirmations to put around the room
- TENS machine
- Some battery operated fairy lights and candles
- Relaxing scents that you've used to anchor during relaxation
- Flannel and fan for keeping cool
- Whatever snacks and drinks you might fancy
- A nice pillow so that you're not stuck with hospital ones
- Hair clips and bands to keep your hair out of your face
- A birth ball just in case all the ones there are in use
- Straw so that you can drink comfortably
- Distraction/something funny to watch or listen to
- An eye mask to get in a little private bubble
- Comfy clothes for during and after birth
- Lots of underwear in case they get messy after birth, big comfy ones!
- Maternity pads- big ones!
- Lip balm
- Nipple cream
- Breast pads
- Clothes, phone charger, underwear etc for birth partner in case you're there for a while
- Toiletries for having a wash after birth
- Change in case that's how you have to pay for parking or for vending machines
- Some slippers or flip flops for walking around your birth place



birth bag checklist

For baby:

- Clothing – pack three or four babygrows , vests and sleepsuits for your little one to wear.
- Going home outfit – quite a few parents have a special going home outfit that they plan to bring their baby home in.
- Scratch mits – these will help prevent your little one from scratching their face with their sharp nails (or suits with mitts included)
- Hat – for when you go outside
- Swaddles – the big wide world is a strange place for your newborn, so give them a sense of security with a swaddle which helps to replicate the feeling of the womb.
- Baby blanket – this will help keep them warm, especially when leaving the hospital.
- Nappies (reusable?) – your newborn will go through a lot of nappies, as many as 12 a day. We always recommend reusable nappies. They help save the planet, your pennies and they're kinder to delicate skin.
- Muslin squares – to mop up any spills or messes easily.
- wipes/cotton wool
- Socks
- Formula/milk if your choice of feeding
- Cord tie



postnatal expectations/planning

After pains

The cramps known as afterbirth pains, or simply afterpains, are caused by contractions of your uterus as it returns to its pre-pregnancy size after you have your baby, and expels any clots. (This process is called "involution.") This can intensify when feeding, but is not usually something to be concerned about, and is often more common after subsequent births. Taking paracetamol or ibuprofen is safe and can help.

Bleeding (AKA Lochia)

It usually begins straight after birth and can continue for up to six weeks postpartum; however, the heaviest flow tends to be during the first two weeks. you may find clots, anything bigger than a 50p should be monitored and anything bigger than your palm should be checked. Between around days four and 10, lochia should get lighter and have a more pinkish or brownish hue. This will still happen following a caesarean.

Never use a tampon!

Swelling

You may experience some vaginal swelling as well as swelling to your vulva. This is quite common, and usually reduces after 2-3 days.

Use 'padsickles' (maternity pad, soaked in aloe and witch hazel and popped into the freezer) to help, plus arnica to reduce any bruising.

Other areas may still have excess water, but this too should reduce in a couple of weeks!

Stitches

If you've had stitches after tearing or an episiotomy, keep them clean by bathing daily, to help prevent infection. Use plain warm water then carefully pat yourself dry. If your stitches are sore or uncomfortable, tell your midwife.

Stitches usually dissolve themselves, but sometimes they have to be taken out - check with your midwife.

Using a Peribottle (or just a jug of warm water) may help reduce any stinging also.

Baby blues

Are due to a sudden drop of hormones, which your body tries to process over the first few days post birth.

Symptoms of baby blues include: feeling emotional, irrational or overwhelmed, tearful (without knowing why), irritable and moody and down or anxious. They usually pass, but if you find they're sticking around, seek support from your midwife or other agencies including PANDAS.

Colostrum

Colostrum is a highly concentrated form of breast milk and is what your breast/chest produces in the first few days after your baby being born. Because it's so concentrated, colostrum often has a thick consistency, and a golden yellow colour.

Colostrum contains high levels of fat, protein and carbohydrates, so your baby will be able to get plenty of nutrition even from the small quantities you produce.*liquidgold*

Going to the loo

You might be shitting yourself (excuse the pun) at the thought of going to the loo post birth.

Try not to hold it, your bladder may have lower levels of control, so go as soon as you feel it. Drink lots to dilute the urine too.

You probably won't have a poo for a few days after the birth, but it's important not to let yourself get constipated. Eat lots of fibre and drink lots of water to help keep your stools soft, occasionally a laxative may be helpful. (p.s. its very unliely your stutches will burst).

Piles

Piles can be quite common (yes I had them).

Try not to push or strain when going to the toilet to help prevent and use 'Anusol' or similar to help the discomfort of they happen. They usually go away within a few days. Be aware of your diet to help your stolls stay soft too.

Night Sweats

Postpartum sweating and night sweats are very common and normal, and they usually go away after a few weeks. Strategies that can help include drinking plenty of water to prevent dehydration, and taking a lukewarm or cool shower before bed.

I recommend all clients do a postpartum plan- the next page is an example and you can find the guide on the hub.





postnatal plan

SOCIAL SUPPORT: WHO CAN PROVIDE YOU SOCIAL SUPPORT? FAMILY, FRIENDS, NEIGHBOURS, DOULAS, OR ONLINE?

- *** parents
- Hypno group
- Leanne
- Peanut App
- (visits, texts, calls, baby groups etc.)
- Night feeding!

REST: HOW WILL YOU MAKE SURE YOU GET ENOUGH REST? HELP WITH HOUSEHOLD, VISITORS, NAPS, RELAXATION TECHNIQUES ETC?

- Lots of rest- *** to support between baby feeds.
- Leanne to support for rest.
- Cleaner/Dog walker to help
- *** parents - Cooks
- Ring doorbell? Door sign? Food drop off? Deliveries?
- Baths and rest, music to switch off, Insta scroll.
- Schedule naps.

EXAMPLE

FOOD: HOW WILL YOU MAKE SURE YOUR BODY IS NOURISHED? BATCH COOK, FOOD DELIVERIES, SUBSCRIPTIONS, SNACKS?

- Batch cooking
- Food vouchers
- Leanne support
- Snack basket (cereal bars, sweets, choc, bananas, dates, nuts)
- Smoothies
- Switch meals (main at lunch)

EMOTIONAL SUPPORT/SELF CARE: HOW WILL ENSURE YOUR RECOVERY PRIORITISES YOU, YOUR HEALING BODY, YOUR EMOTIONAL NEEDS, FRIENDS, FAMILY, PHYSIOTHERAPIST, MASSAGE TECHNIQUES, POSTNATAL SUPPORT?

- Self care: moving, walk around block, being outside, showers
- Do make up
- Postnatal support massage
- Leanne - postpartum yoga support.
- If want massage/hair etc, can use Leanne to support with baby etc.

Think about-

- You and your baby's immediate needs
- You and your baby's secondary needs

writing your birth story

Writing your birth story can be a really lovely thing to do after a positive birth, it can also be really healing when birth didn't go as you hoped. You can do it anytime at all after your baby is born. If you'd like to do it when your baby is newborn or 20 years old, crack on!

Here are some prompts to help you include everything....

How was your pregnancy?

Did you have any particular pregnancy symptoms? Any special circumstances for your- self or baby? How did you feel about labour and birth? What antenatal education or reading did you do to prepare? What type of birth were you planning? How did you feel in the days before baby was born? Were there any signs labour was coming?

How did labour begin?

Did your waters break in the middle of the supermarket? Did you think you had the shits because of the sensations? Were you induced?

How did you spend early labour?

What did you do to keep comfortable? Did you do any activities? Did you go out an- ywhere? Did you have anything yummy to eat? Who was with you? Did you watch or listen to anything?

What happened during active labour?

How did you recognise it was time to head to your birth space or call for a midwife to attend? How was the journey? How did you feel? Did setting up your birth space go well? What did you use to keep comfortable? How was your progress assessed? Did you have any vaginal exams or observations? Did you use a birth pool or have any pain relief? What did your birth partner/s do that was helpful? How were the staff?

The end of labour...

Did you feel a change in intensity? Were interventions offered? Did you feel like everything was explained to you and your decisions respected? Did you or your birth partner recognise the transition phase of labour? Did you feel the urge to push? What position were you in as baby was born? Was your baby born in a theatre with forceps or caesarean? How did that change to your birth plan feel?

Meeting your baby

What were your first thoughts as you met your baby face to face for the first time? Who was the first person to touch your baby. How were baby and you immediately after birth? How did you birth the placenta? Did you change locations after baby was born? (getting out of a pool, laying on a bed, moving rooms or to a ward etc). Did you have any stitches? Were you already home or how long did you stay in hospital if relevant.

That should cover all the basics to help prompt you to remember the various parts, you may feel like you need to ask someone else who was there about the details, that's perfect- ly normal!

Your birth story can be just for you to treasure or you could share it with the world!

Positive birth stories can really help others who are preparing for their own birth.

Notes:



useful resources

NICE Guidelines- These are the recommendations for maternity care and are a good base to start. As always these are guidelines and not laws. These can be found online.

www.nice.org.uk/guidance

Evidence Based Birth- Easy to understand articles that bring together all the evidence from studies on different topics in the one place. Really interesting stuff on due dates, 'big' babies and gestational diabetes amongst other topics.

www.evidencebasedbirth.com

Sara Wickham- A fantastic Dr, Midwife and researcher who has written lots of easy to digest books on various topics such as inducing labour, birthing the placenta and group B strep and many more. Plenty of info on birth that's useful to everyone.

www.sarawickham.com

Spinning Babies- Loads of information on baby's positioning, resources to help to aid baby into the optimal position for birth and exercises to move baby if you wish to. Great for turn- ing breech babies!

www.spinningbabies.com

AIMS (Association for Improvements in the Maternity Services)- Loads of unbiased in- formation about birth. There is a helpline to get support and information on your specific circumstances.

www.aims.org.uk

Tell Me A Good Birth Story- Read positive birth stories to fill your mind with how fantastic birth can be! There are birth stories on ALL forms of birth!

www.tellmeagoodbirthstory.com

Birth Rights UK- This is a charity that aims to improve the way people are treated during pregnancy and birth and they are a great place to go if you think you are being denied something that you are entitled to eg. birth pool, home birth etc.

www.birtherights.org.uk

Midwife Thinking Blog- This is a blog with various informative posts by Rachel Reed, author of Why Induction Matters, she is a midwife and lecturer who aims to share evidence based information with parents.

www.midwifethinking.com

La Leche League- Information on all aspects of breastfeeding. Look for your local LLL Facebook group or in person meetings for local BF support.

www.lalecheleague.org.uk

Wendy Jones at The Breastfeeding Network Drugs in Breastmilk information service- If you are prescribed a medication that is said to not be compatible with breastfeeding then always double check this with Wendy. You can send a message on facebook and receive a reply very quickly about your specific situation. They also have fact sheets on various medications and conditions.

www.breastfeedingnetwork.org.uk

Sarah Ockwell Smith- A fantastic parenting expert who explains all about how childrens brains change and grow and how we can support them and manage our expectations. She is also an author with some books on our reading list!

www.sarahockwell-smith.com

Timeline of a Breastfed Baby- An amazing way of finding out what is usual for a breastfed baby at birth and each day/ week/ month after that! Also a great cheerleader as it tells what breastfeeding has done for your baby in that time.

www.thealphaparent.com/timeline-of-a-breastfed-baby/

Wonder Weeks- A great resource for finding out more about the different things your baby is learning and what behaviour you might expect. Phone app is available for a small fee and is well worth it.

www.thewonderweeks.com

Birth Kweens Podcast- This is an american podcast hosted by a midwife and a doula. It's easy listening and full of information, there are various guests such as health professionals and people telling their birth stories etc.

Available wherever you listen to podcasts eg. Spotify.

The Business of Being Born- American documentary created by Ricki Lake discussing the medicalisation of childbirth in America but there are lots of parallels to the UK.

Available on Amazon Video with a free trial of Gaia (2020).

Why Not Home?- A documentary about healthcare professionals talking about why they chose home birth. Great for those deciding on their birth place to look into reasons why you might choose home birth and things that you may find in hospital so that you can get ideas of how to combat them.

~Can be rented on the website: www.whynothome.com

The Birth Uprising Instagram- @thebirthuprising Loads of information on birth, your rights and stories from other parents. The stories are always full of shares to other great birth related instas so it's a great place to start to start seeing accurate and positive info about birth on your feed!

www.instagram.com/thebirthuprising

AND FINALLY!!!

My birthy podcast 'Dou-La-La' where we discuss a range of topics with an evidence based approach in an informal and chatty way.

<https://www.birthing-hood.co.uk/doulapodcast>



recommended reading list

- Anything from the 'Why It Matters' series published by Pinter and Martin
- The First forty Days: The Essential Art of Nourishing a New Mother by Heng Ou The Womanly Art of Breastfeeding by La Leche League International
- The Positive Breastfeeding Book by Dr Amy Brown
- The Politics of Breastfeeding by Gabrielle Palmer
- The Book You Wish Your Parents Had Read by Philippa Perry
- The Gentle Sleep Book by Sarah Ockwell-Smith
- Baby Calm and Toddler Calm by Sarah Ockwell-Smith
- Baby Led Weaning by Gill Rapley
- I Am Not Your Baby Mother by Candice Brathwaite
- Informed Is Best by Amy Brown
- Vitamin K & The Newborn by Sara Wickham
- Inducing Labour by Sara Wickham
- Group B Strep Explained by Sara Wickham
- The Birth Partner by Penny Simkin
- Am I Allowed by AIMS
- Breastfeeding and the Fourth Trimester by Lucy Webber

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